Angela, a patient living with ASCVD + CKD and a recent heart failure diagnosis

Patients with ASCVD and hsCRP ≥2 mg/L have an increased risk of MACE (30%), hospitalisation for heart failure (24%) and all-cause death (35%), versus patients with hsCRP <2 mg/L.[1*]

Angela, 76.

ASCVD + CKD

6 Months post-HFpEF diagnosis

LVEF: 55% NYHA class II

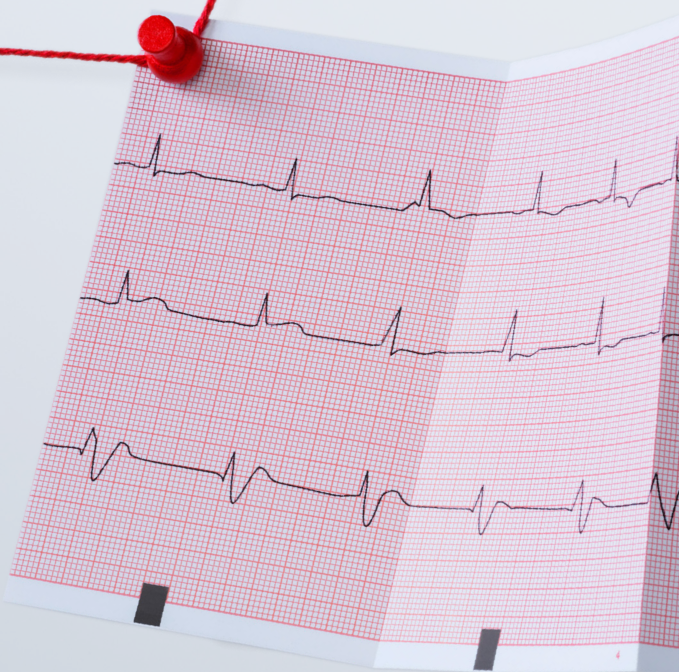
LDL-C: 58 mg/dL

eGFR: 42 mL/min/1.73m^2

Current Tx: Anti-platelet, Intensified statin therapy, RAASi, Beta blockers, SGLT2i, Diuretic

- Angela has been stressed recently; she's sleeping poorly and has a diet high in processed foods. She takes short walks but struggles with anything longer than twenty minutes. She still smokes occasionally.
- At first glance, her treatments seem to be working. Her LDL-C has improved, and her EF seems to be stable.
- You recently attended a conference and remember reading: **even in patients on high-intensity lipid lowering therapy, 1 in 2 CVD patients are still facing inflammatory CV risk.**[2]
- **Inflammatory risk can be detected using the prognostic biomarker hsCRP (≥2 mg/L).**[1–3]
- You decide to check Angela's hsCRP levels. The result comes back:

  2.9 mg/L.
- **You check the 2025 ACC Scientific Statement on inflammation in CVD. It highlights that hsCRP ≥2 mg/L signals an increased risk for recurrent CV events, irrespective of LDL-C levels.**[4]



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- You explain to her that CV inflammation is a key driver of her heart failure symptoms.[5]
- She's already taking intensified statin therapy, so you decide to focus on improving her lifestyle.
- You explain to Angela that she's at additional CV risk because of CV inflammation, but she can reduce some of her overall CV risk via lifestyle choices. She listens carefully and says she's finally ready to stop smoking.

Are you using hsCRP levels to guide meaningful patient discussions and optimise CV risk management?

Footnotes:
Adjusted for age, sex, time since ASCVD, eGFR, albuminuria, comorbidities undertaken procedures, and ongoing medications. Based on an observational cohort of 84,399 adults with ASCVD undergoing routine hsCRP testing. Over a median 6.4-year follow-up, the adjusted relative rate of events in participants with hsCRP ≥2 mg/L (vs hsCRP <2 mg/L) was 30% higher for MACE (adjusted HR 1.30; 95% CI 1.27–1.33), 24% higher for heart failure (HR 1.24; 95% CI 1.20–1.30), and 35% higher for all-cause death (HR 1.35; 95% CI 1.31–1.39).[1]

References:
1: Mazhar F, et al. Eur Heart J 2024; 45(44):4719–4730.
2: Ridker PM. J Am Coll Cardiol 2018; 72:3320–3333.
3: Ridker PM et al. Lancet 2023; 401(10384):1293–1301.
4: Mensah GA, et al. J Am Coll Cardiol. 2025; 0(0).
5: Kurt B, et al. Curr Heart Fail Rep 2025; 22(35):1–17

Abbreviations:
ACC=American College of Cardiology, ASCVD=Atherosclerotic cardiovascular disease, CKD=Chronic kidney disease, CV=Cardiovascular, eGFR=Estimated glomerular filtration rate, HFpEF=Heart failure with preserved ejection fraction, hsCRP=High-sensitivity C-reactive protein, LDL-C=Low-density lipoprotein cholesterol, LVEF=Left ventricular ejection fraction, NYHA=New York Heart Association, Tx=Treatment.

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